

HOW TO OPEN A GROUP HOME BUSINESS in Georgia

Host homes & community residential alternatives under the NOW & COMP waivers.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Georgia model — read this first

“Group home” in Georgia means providing residential support to adults with intellectual and developmental disabilities, and the state funds it through two 1915(c) waivers — the New Options Waiver (NOW) and the Comprehensive Supports Waiver Program (COMP) — run by the Department of Behavioral Health and Developmental Disabilities (DBHDD). Two things make Georgia distinctive, and they run through everything below. First, the residential service has a specific name: COMMUNITY RESIDENTIAL ALTERNATIVE (CRA), and it comes in two sizes — a host home (life-sharing) for one or two adults, and a group home for three or four adults in a licensed setting. Residential CRA is a COMP service. Second, TWO STATE AGENCIES stand between you and a paid claim: DBHDD enrolls and standards-checks you, while the Department of Community Health (DCH) — Georgia's Medicaid agency — issues your provider number and pays your claims through GAMMIS. Because Georgia's rules and the size of a real group-home operation point to an established structure, the realistic opening for many NEW providers is a host home under an existing CRA agency, growing into your own agency.

The two ways in. Path A — become a host home / life-sharing site under an existing CRA provider agency. One or two adults live in your home; the agency enrolls your site, supports you, and handles the billing. The Medicaid provider number belongs to the AGENCY, not to you. Smallest, fastest, closest to a host home. Path B — become your OWN CRA provider agency (enroll with DBHDD through the Georgia Collaborative ASO, enroll with DCH/Medicaid, license your group homes as Community Living Arrangements, and employ a Developmental Disabilities Professional and staff). A much bigger lift — but it's the real business. This guide walks both, and is honest about which is realistic when.

The names you'll deal with. DBHDD = Department of Behavioral Health and Developmental Disabilities (runs the NOW & COMP waivers, enrolls providers, sets standards). DCH = Department of Community Health, Georgia's Medicaid agency (issues your Medicaid provider number, pays claims through GAMMIS). Georgia Collaborative ASO = the administrative services organization that runs provider enrollment. Support Coordinator = the person who develops the Individual Service Plan (ISP) and coordinates services. Regional Field Office = one of six DBHDD offices that determines waiver eligibility and is your first local point of contact. HFRD = Healthcare Facility Regulation, which licenses group homes as Community Living Arrangements (CLAs).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or billing advice; requirements, rates, and codes change quarterly, so verify each step with DBHDD, the Georgia Collaborative ASO, DCH, HFRD, your local fire authority, and qualified local counsel (see Part 12) before you rely on it.

The journey at a glance

These milestones take you from idea to your first payment — whether you start as a host home or build your own agency.

1	Decide your path & set up the business	Path A (host home) or Path B (CRA agency); entity, EIN, NPI, insurance (Parts 3–4)
2	Learn the CRA standards	Read the COMP waiver CRA requirements and the DBHDD provider manual (Part 5)
3	Enroll with DBHDD	Enrollment Forum, Letter of Intent, provider application & site visit via the Georgia Collaborative ASO (Part 5)
4	Enroll with Medicaid (DCH)	Complete the DCH Medicaid application after DBHDD recommends you (Part 5)
5	License your group homes	Each CLA licensed by HFRD under rule 290-9-37 (Part 6)
6	Ready the home	Settings/rights, the 7-bed sprinkler rule, and fire safety (Part 6)
7	Staff & write your policies	The DDP, direct support professionals, RN oversight, documentation (Parts 7–8)
8	Get a person placed	Planning List, COMP slot, ISP, match & prior authorization (Part 9)
9	Deliver services & get paid	Bill Georgia Medicaid through GAMMIS against the approved PA (Part 10)

Rule of thumb. Start smaller than you think. A brand-new four-bed licensed group home is a large lift for a first-time provider — the CLA license, the physical plant, the DDP, and the staffing all come at once. Many people begin as a host home under an established CRA agency, learn the system on someone else's provider number, and grow into their own agency once they know the terrain.

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is a relationships-and-documentation business funded by Medicaid, not a real-estate play or a normal small business: your revenue depends on getting enrolled by DBHDD, credentialed by DCH, and licensed by HFRD, and then on a person actually being placed with you — so your reputation with DBHDD, established providers, Support Coordinators, and the Regional Field Office is your most valuable asset. Second, the money arrives on a chain and on a delay: the DBHDD enrollment process runs through an Enrollment Forum and a Letter of Intent during an open window, then a provider application and site visit; DCH Medicaid enrollment comes on top of that (and a DCH denial carries a one-year wait to reapply); and you don't bill until a person is placed and their service is prior-authorized — so you need patience and a working-capital cushion to carry any staff and setup costs through the ramp. Start on Path A as a host home under an established provider if you can, learn the system on someone else's provider number, and grow into your own agency once you know the terrain.

Here's how the rest of this guide is organized: Parts 1–2 explain what the business is and who regulates and pays you; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 walks the two paths in; Part 5 is the DBHDD-enrollment-then-DCH-enrollment chain; Part 6 covers the home, the CLA license, and fire safety; Parts 7–8 are staffing and the policies and records DBHDD expects; Part 9 is how a person actually gets placed with you; Part 10 is how you get paid; Part 11 covers costs, timeline, and getting placements; and Part 12 collects every phone number and script in one place. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Providing residential support (Community Residential Alternative) to adults with developmental disabilities under the NOW & COMP waivers — as a host home for one or two adults, or a group home for three or four. Paid by Georgia Medicaid through GAMMIS, after DBHDD enrolls you and DCH credentials you.*

A “group home” business in Georgia is really a developmental-disabilities support business. You help an adult with an intellectual or developmental disability live successfully in the community — with daily support, supervision, skill-building, help with health and medications, and a real home life — rather than in an institution. Georgia's residential waiver service for this is Community Residential Alternative (CRA), and it spans a spectrum: from a host home (life-sharing) where one or two adults share your family's everyday life, to a licensed group home for three or four adults staffed by direct support professionals.

Be clear-eyed about what kind of business this is. This is a human-services business built on relationships and documentation, funded by Medicaid — not a real-estate play. Your value is the quality and reliability of the support you provide and your ability to keep it compliant and well-documented, because that's what DBHDD enrolls, what DCH pays for, what HFRD licenses, and what keeps people safe. Go in understanding that the care and the paperwork ARE the business.

Why this business — and why now

Demand for community-based services for people with developmental disabilities is high and durable: Georgia, like every state, is moving people out of institutions and into the community, families overwhelmingly want a real home for their loved one, and the NOW & COMP waivers fund exactly this. Because eligible people sit on the I/DD Planning List and are matched to providers as slots open, a reliable, well-run provider that builds relationships with Support Coordinators and Regional Field Office staff can keep its homes full. And you can start lean — as a host home under an established CRA agency — and grow into your own agency as you learn the system.

Be honest about the two realities that trip up new providers. (1) *Two agencies, not one: you don't just get a state license and bill Medicaid — you get enrolled by DBHDD AND credentialed by DCH before a dollar flows, and each CRA site is enrolled separately.* (2) *Room and board is separate from your pay: the CRA rate pays for the SERVICE, not the person's rent and food, which come from their own income (usually SSI).* Plan your business — and explain it to families — around both.

PART 2

Who Regulates You & Who Pays You

In short: *Three bodies stand between you and a paid claim: DBHDD enrolls and standards-checks you, DCH credentials you as a Medicaid provider and pays your claims through GAMMIS, and HFRD licenses your group homes. Your Regional Field Office and Support Coordinators build each person's plan and eligibility.*

DBHDD — Department of Behavioral Health and Developmental Disabilities (your enroller & standard-setter)

DBHDD runs the NOW & COMP waivers day-to-day, sets the CRA standards, enrolls providers, and monitors them. Enrollment runs through the Georgia Collaborative ASO — you attend a New Provider Enrollment Forum, submit a Letter of Intent during an open window, then complete the DBHDD New Provider Application, which is guided by DBHDD policy 02-701. Host-home enrolling, matching, and monitoring is guided by DBHDD policy 02-704. Without DBHDD enrollment you cannot provide CRA services.

DCH — Department of Community Health / Medicaid (your credentialer & payer)

After DBHDD approves you and forwards a recommendation, you complete the online DCH Medicaid application. DCH issues your Medicaid provider number — which is service- and site-specific, so each CRA site is enrolled separately — and pays your claims through GAMMIS (mmis.georgia.gov). Important: if DCH denies your application, you must wait one year to reapply, so get it right the first time.

HFRD — Healthcare Facility Regulation (your licensing body for group homes)

A group home serving two or more DBHDD-funded adults must be licensed by HFRD as a Community Living Arrangement (CLA), under Georgia Rules & Regs 290-9-37. HFRD inspects the home before licensure, then periodically and on any complaint. A host home for one person is a private residence enrolled site-specifically under your CRA agency — lighter than a full CLA. Your Support Coordinator and the DBHDD Regional Field Office handle each person's eligibility, ISP, and placement.

What this means for you: the path to payment is a chain — DBHDD (enroll) → DCH (credential, pay via GAMMIS), with HFRD (license the group home) in parallel — and each link takes time. Start them in the right order and in parallel where you can, and treat the Regional Field Office, Support Coordinators, and established CRA agencies as key relationships, because that's how people get matched to your home.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Georgia you form the LLC through the Secretary of State. Your entity, EIN, and NPI all have to be in place and consistent before DBHDD enrollment and DCH Medicaid enrollment, because they're checked along the way — and your Secretary of State registration is also required for the CLA license. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the Georgia Secretary of State.** Online through the eCorp system at sos.ga.gov — the LLC filing fee is \$100 online (\$110 by paper). Name the LLC and list a registered agent with a Georgia street address (this can be you).
- 2. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your DBHDD enrollment file, your CLA license application, and your Medicaid enrollment will all reference your structure.
- 3. File the annual registration.** Georgia LLCs file an annual registration with the Secretary of State — the fee is \$50 per year (due by April 1) to stay in good standing.
- 4. Confirm local business licenses.** Georgia issues most local business licenses (occupational tax certificates) at the city or county level — check what applies where you'll operate.

No newspaper publication in Georgia. Georgia has no LLC publication requirement — forming the entity is a straightforward Secretary of State filing (\$100) plus the \$50 annual registration. Keep your registration current, because your Secretary of State standing is checked at CLA licensing.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with DCH as a Georgia Medicaid provider and to bill CRA services through GAMMIS under the NOW & COMP waivers. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

2. Start a Type 2 (Organizational) application. Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a developmental-disabilities / community-based residential provider — confirm the exact one DCH and DBHDD want for CRA billing. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Georgia branches — Truist, Bank of America, Wells Fargo, Synovus, Ameris Bank, or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A CRA provider brings staff into people's homes and supports vulnerable adults, so you need coverage built for a human-services / developmental-disabilities provider — and DBHDD and HFRD will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the SUPPORT and supervision you provide	Yes — foundational
Abuse & Molestation	Allegations of abuse of a person you support — general liability EXCLUDES this	Yes — high exposure
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended for an agency
Non-Owned & Hired Auto	DSPs driving their own cars to support or transport the people you serve	Yes — critical
Workers' Compensation	DSP injuries	Required in GA (3+ employees)
Property	Coverage on any provider-owned or leased group home	If you own/lease a home

Abuse & molestation is your highest exposure. Supporting vulnerable adults means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. Confirm it explicitly.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities provider programs — this is a specialty niche. Carriers and programs active in DD/IDD services include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local Georgia agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for a developmental-disabilities / CRA-provider program. Confirm the carrier writes this in Georgia before you rely on a quote.

What to ask the insurance agent

“Do you write developmental-disabilities / CRA providers in Georgia — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Do you include non-owned & hired auto for my DSPs driving the people we support?”

“What limits do DBHDD and HFRD expect for a CLA?”

“Can you set up Georgia workers' comp for my direct support professionals?”

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves Georgia providers real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your DBHDD provider application, your HFRD CLA license, and your DCH Medicaid enrollment. Your information passes through three organizations before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there — is one of the most common and most frustrating causes of a stalled enrollment or a rejected claim. This matters even more in Georgia because a DCH denial carries a one-year wait to reapply, so a sloppy application isn't just a delay — it can cost you a year. Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It's worth an hour with your accountant or attorney up front to confirm the structure. And keep your Secretary of State registration, DBHDD enrollment, CLA license, and Medicaid credentialing calendared for renewal — letting any one of them lapse stops your payments even when the care is going perfectly.

The business foundation to have in hand before you apply for enrollment:

- Your formed Georgia LLC (Articles of Organization filed, \$50 annual registration current) and Operating Agreement
- Your EIN confirmation (CP-575) from the IRS
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity's exact legal name
- Bound insurance — general/professional liability, abuse & molestation, non-owned auto, and workers' comp
- A single, consistent legal name, ownership list, and address used identically on every document

With those six things in place and consistent, you're ready to build the DBHDD enrollment application in Part 5 on a clean foundation — and you've removed the paperwork mismatches that stall more Georgia applications than any substantive problem does.

PART 4

The Two Paths — Host Home (A) or CRA Agency (B)

In short: Path A is the fast, small way in — become a host home / life-sharing site under an established CRA agency. Path B is the real business — become your own CRA provider agency. Pick the one that fits where you are today; many start with A and grow into B.

4.1 Path A — a host home / life-sharing site

A host home (life-sharing) means one or two adults with developmental disabilities live in your home and share your everyday life. They can't be related to you by blood or marriage, and you can't be an employee of the CRA agency you contract with. Everyone in the household is part of the screening — this is a family decision, and it's the fastest, simplest way in.

- 1. Make sure it fits your life.** Everyone in the household is part of this, so it's a family decision — go in with eyes open about the good days and the hard ones. A host home for one person is small and simple; two adults may add requirements.
- 2. Choose a CRA provider agency.** On this path you don't enroll with the state yourself — you partner with a DBHDD-approved CRA provider agency that enrolls your home (each host home requires its own site-specific enrollment), supports and monitors you, and handles the billing. Interview more than one; ask how they support and pay host homes.
- 3. Pass background checks & training.** Complete a criminal-history background check and the training DBHDD and the agency require (including the HCBS Settings Rule and health-and-safety topics) before the agency applies for your site-specific Medicaid provider number. The agency follows DBHDD policy 02-704.
- 4. Get your home ready, then get matched.** Your home passes the health-and-safety requirements for a host-home site (see Part 6); then the agency, the person's Support Coordinator, and the Regional Field Office match you with someone whose needs and personality fit your home — the match is the heart of life-sharing. You'll sign an attestation and agreement with the agency.

The difficulty-of-care tax note. Because the person you support lives in your OWN home, your host-home payments may qualify for the IRS “difficulty-of-care” exclusion (potentially non-taxable). Confirm with a tax professional — it can meaningfully change the economics of a host-home arrangement.

4.2 Path B — become your own CRA provider agency

Path B is more work, more money, and more control: you enroll with DBHDD through the Georgia Collaborative ASO, complete DCH Medicaid enrollment, license your group homes as CLAs through HFRD, employ a Developmental Disabilities Professional (DDP) and staff, and can recruit your own host homes. It's the real, scalable business — and Part 5 walks the enrollment path step by step. Most operators who go this route either have direct experience in DD services or hire someone who does, because your application has to show DBHDD you understand the standards and can deliver.

PART 5

Enroll with DBHDD, Get Credentialed by DCH & License Your Homes

In short: For Path B, this is the chain that unlocks payment: learn the CRA standards, enroll with DBHDD through the Georgia Collaborative ASO (Enrollment Forum → Letter of Intent → application → site visit), complete DCH Medicaid enrollment, and license each group home as a CLA through HFRD.

- 1. Learn the standards cold.** Read the COMP waiver CRA service requirements (Part III, Chapter 1700, on GAMMIS), the DBHDD Provider Manual for Community DD Providers, and the NOW/COMP fee schedule closely — your application must show you understand them and describe HOW you'll deliver services.
- 2. Attend the Enrollment Forum & submit your Letter of Intent.** Enrollment runs through the Georgia Collaborative ASO (georgiacollaborative.com). Register for and attend a New Provider Enrollment Forum, then submit an I/DD Agency Provider Letter of Intent (LOI) during an open enrollment period. Once the LOI is approved, you're invited to apply.
- 3. Submit your DBHDD provider application (and site visit).** Complete the DBHDD New Provider Application for the services and sites you'll offer. DBHDD may conduct a site visit, verifies you meet the criteria, and forwards a recommendation to DCH. This is guided by DBHDD policy 02-701.
- 4. Complete Medicaid (DCH) enrollment.** After DBHDD approves, complete the online DCH Medicaid application. Medicaid provider numbers are service- and site-specific, so each CRA site is enrolled separately. If DCH denies, you must wait one year to reapply — so get it right the first time.
- 5. License your group homes & complete orientation.** License each group home as a CLA through HFRD (Part 6), carry your insurance, put your policies in place, hire and train staff, and complete New Provider Orientation. Then you can accept referrals, deliver CRA services, and bill.

What this means for you: enrollment is a quality gate, not a form. DBHDD wants evidence — real policies, real service descriptions, real capacity, and a DDP — that you can safely support people. The providers who get through cleanly treat the CRA standards and the provider manual as the blueprint for the whole operation, so building your policies and staffing to the standard does double duty: it earns your enrollment AND it's how you'll actually run.

PART 6

The Home — the CLA License, Settings, Rights & Fire Safety

In short: A group home for two or more DBHDD-funded adults must be licensed as a Community Living Arrangement (CLA) by HFRD under rule 290-9-37. A host home for one person is a lighter, site-specific enrollment. Georgia's fire-sprinkler rule is refreshingly specific — sprinklers only at seven or more beds. In every case, it must genuinely be the person's home.

6.1 How Georgia handles the home — CLA vs. host home

A group home serving two or more DBHDD-funded adults must be licensed as a Community Living Arrangement (CLA) by Healthcare Facility Regulation under Georgia Rules & Regs 290-9-37. A host home serving one person is a private residence enrolled site-specifically under your CRA agency — lighter than a full CLA. Which you're opening changes what your house has to be. In every case, the home must genuinely be the person's home, with the HCBS Settings Rule rights — a lease or residency agreement, privacy, choice, and full access to the community.

6.2 “Do I need fire sprinklers?” — Georgia gives a clear answer

For a licensed CLA, the rule is refreshingly specific: a fire-sprinkler system is required for a home with SEVEN OR MORE beds. A CRA group home (three or four residents) or a host home (one or two) is well under that, so a sprinkler system generally is NOT required. What you do need is a fire-safety inspection report from the appropriate fire authority (or State Fire Marshal), dated within 12 months, with no violations and showing the capacity load. Confirm anything unusual with your local fire authority BEFORE you buy or renovate.

6.3 The CLA home checklist (rule 290-9-37)

- ☐ A fire-safety inspection report (within 12 months, no violations, showing capacity load) — sprinklers only if 7+ beds.
- ☐ A floor sketch labeling rooms, doors, windows, and bed placement, with square footage.
- ☐ Approved water and sewage — a public water/sewer bill, or a County Environmentalist report for well/septic.
- ☐ Georgia Secretary of State registration for your entity, proof of ownership or lease, and background checks for owners (10%+ interest).
- ☐ HCBS Settings Rule rights honored — a lease or residency agreement, privacy, choice, and full community access.
- ☐ HFRD inspects before licensure, then periodically and on any complaint — and you consent to unannounced visits.

Two things people get wrong. (1) Zoning — a host home is a private residence in a residential area, and small homes for people with disabilities are protected as residential use under the federal Fair Housing Act; still, confirm your address and any local rules with your city or county. (2) Room and board is separate from your pay — the CRA rate is for the SERVICE, not the resident's rent and food (those come from their own income, usually SSI). Keep the two completely separate in your books and your agreements.

PART 7

Who You Actually Need — Staffing

In short: As a host home (Path A), you and your household provide the support, with the CRA agency's professional oversight, DDP, and billing. As a CRA agency (Path B), plan for a director, a required Developmental Disabilities Professional, direct support professionals, nurse oversight, and billing — one person can wear several hats at the start.

7.1 The roles

Host home (Path A): you and your household are the support. The CRA agency provides the professional oversight, the DDP, and the billing, and helps arrange respite/relief. You don't build a staff. As a CRA provider agency (Path B), plan for these roles:

- **Agency director / administrator:** runs the agency and owns compliance and the DBHDD/DCH enrollment.
- **Developmental Disabilities Professional (DDP):** the required clinical/professional role — must be primarily employed by your agency with enough hours to meet participants' needs. This is a Georgia-specific requirement DBHDD checks.
- **Direct support professionals (DSPs):** the caregivers in your group homes — each background-checked and trained.
- **Registered Nurse:** for health oversight, medication, and nursing (RNs/LPNs need IDD Healthcare CEUs) — often part-time or contracted at first.
- **Billing / administrative support:** handles GAMMIS billing, prior authorizations, and documentation.

7.2 Recruiting and keeping DSPs — the real make-or-break

Direct support professionals are the heart of this business and the hardest thing to keep. DSP turnover is high across the whole field, and a home that can't reliably staff its shifts can't safely support people or keep its authorizations — so treat recruiting and retention as your core operation, not an afterthought.

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability.
- **Onboard to the standard:** real orientation, the required background checks and training (including HCBS Settings Rule and health-and-safety topics), and a competency check before a DSP supports someone alone — this protects the people you serve and satisfies DBHDD.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a DSP hits a hard day, and recognition. Every DSP who quits costs you re-hiring, re-training, and often continuity for the person they supported.

PART 8

Policies, Records & Staying Compliant

In short: *Your policies and documentation are what DBHDD enrolls against and what HFRD and DCH can audit. Build them to the CRA standard from day one — medications, emergencies, incident reporting, and the rights of the people you support.*

You keep clear documentation covering medications, emergencies, incident reporting, and the rights of the person you support — and the Individual Service Plan (ISP) drives what you deliver and record. This documentation is both the DBHDD enrollment requirement and the backbone of a safe, well-run operation. PolicyReady's Georgia manuals are built for exactly this: ready to customize, just add your details. Build a simple system that keeps each person's ISP, service records, and incident documentation current, because DBHDD monitors it, HFRD inspects it, and DCH can audit it.

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

The Person's Journey — Planning List to Move-In

In short: *You can't get paid until someone is placed with you and their CRA service is prior-authorized. The path runs from eligibility and the I/DD Planning List, through a COMP waiver slot and the ISP, to the match and prior authorization.*

- 1. The person qualifies — and often waits.** They have an intellectual disability (originating before age 18) or a related developmental disability (before age 22), meet an ICF level of care, and are Medicaid-eligible. A DBHDD Regional Field Office determines waiver eligibility.
- 2. They get COMP funding.** Residential CRA is a COMP service. Because waiver slots are limited, eligible people are placed on the I/DD Planning List and prioritized by unmet need.
- 3. The plan is built.** Once funded, a Regional Planning List Administrator and Support Coordinator help the person and family choose providers and develop the Individual Service Plan (ISP).
- 4. The match and prior authorization.** You (or your CRA agency) are matched to the person; the CRA service and rate are prior-authorized in the Medicaid system BEFORE services start.
- 5. Move-in.** The person moves in and, with services authorized, you're delivering a billable CRA service.

What this means for you: because slots are limited and people are matched based on fit and choice, your census depends on relationships and reputation, not marketing. Get known to your Regional Field Office, to established CRA agencies, and above all to Support Coordinators as the provider who supports people well, communicates, and handles a placement smoothly — and the matches follow.

Plan your business around the Planning List reality. *The I/DD Planning List is long, and COMP slots open on their own schedule — which shapes how you should plan. Don't build a business model that assumes you'll fill four beds the month you're enrolled; assume a gradual build as Support Coordinators learn to trust you and matches come through. Two practical moves help: start on Path A as a host home under an established agency so you have revenue while you build, and stay in regular, helpful contact with your Regional Field Office and the Support Coordinators in your area so that when one of their people is choosing a provider, you're the name they think of first. Patience plus reliability is the whole strategy — the demand is real, but it arrives one authorized person at a time.*

PART 10

How You Get Paid — the CRA Per-Diem & Your First Payment

In short: CRA is paid as a per-diem (daily) rate, tiered by the resident's assessed needs, with separate rate models for host homes and group homes. Every service must be prior-authorized and billed through GAMMIS. Room and board is separate.

10.1 How the rate works

Community Residential Alternative is paid as a per-diem (daily) rate — and Georgia sets different rate models for host home services and group home services. Within each, the rate is tiered by the resident's assessed needs, and for group homes it also reflects the home size (a three-person vs. a four-person residence). Higher acuity means a higher tier and a higher rate.

- **Host home vs. group home:** separate per-diem rate models — a host home (1–2 people) is priced differently from a group home (3–4).
- **Tier (acuity):** CRA rates step up by tier (roughly Tier 1 through Tier 4) based on the resident's assessed level of need.
- **Home size:** group home rates differ for a 3-person vs. a 4-person residence.
- **Extra staffing:** when a person needs hours beyond the rate-model assumptions, a separate Additional Residential Staffing service (basic or enhanced) can be authorized.
- **Where to look:** the exact procedure codes and current per-diem amounts are in the COMP fee schedule on GAMMIS (mmis.georgia.gov). Confirm the current figure before you budget.

10.2 The money math — how CRA pencils out

CRA economics are different from an hourly private-pay business — your revenue is the per-diem for each authorized resident, and your biggest cost is DSP labor. Lay it out with your own numbers:

- **Revenue:** the authorized CRA per-diem for each resident (host-home or group-home rate model, at the resident's tier), billed to Georgia Medicaid through GAMMIS, plus any authorized Additional Residential Staffing.
- **Cost:** DSP wages and payroll taxes are the bulk; then RN and DDP oversight, administration, insurance, and (for a group home) the building and the CLA/physical-plant requirements. Most are fixed against your authorized census.
- **The lever:** because the per-diem is set by the waiver and tiered by acuity, your margin comes from staffing efficiently to each resident's authorized plan and keeping your beds filled with authorized people — utilization, not rate, is the number you manage.
- **Room and board stays separate:** the resident's rent and food come from their own income (usually SSI), NOT the waiver — keep that money and those agreements entirely separate from your CRA billing, and never bill the person for a covered service.

10.3 How you bill — and one important rule

- **Prior authorization first:** every waiver service must be prior-authorized (PA) in the Medicaid system, and claims pay only according to the approved PA — confirm the PA is in place before you deliver services.
- **Billed through GAMMIS:** claims go to Georgia Medicaid via GAMMIS (a host home's claims are filed by the CRA agency).
- **Room and board is separate:** the CRA rate doesn't include it, and you never bill the person for a covered service.

10.4 Your first payment

1. **Confirm the prior authorization matches the CRA service, tier, and site.**

2. Deliver the service and keep your daily documentation.

3. Submit the claim through GAMMIS. The CRA agency does this for a host home; otherwise you bill per your DCH enrollment.

4. Get paid — and keep everything current. Claims pay per the approved PA, so keep authorizations, enrollment, and your CLA license current so payment never lapses.

10.5 Keeping your claims clean — the Georgia pitfalls to avoid

Most CRA payment problems in Georgia aren't about the care — they're about the paperwork chain that sits behind every claim. A few habits keep your GAMMIS claims paying cleanly and keep you off DBHDD's and DCH's radar:

- **Bill only against a live prior authorization:** CRA is a per-diem, so a claim that runs past the authorized dates, exceeds the authorized tier, or names the wrong site will deny — check that the PA covers every day you bill, at the tier and site on the ISP, before you submit.
- **Keep the per-diem and the room-and-board money in separate accounts:** the resident's SSI-funded rent and food never touch your CRA billing; commingling the two is one of the fastest ways to a payback finding, and it confuses residents and families about what they owe.
- **Match every claim to your daily documentation:** the per-diem is earned by the service actually delivered and recorded that day — if a DBHDD monitor or DCH auditor asks, your daily notes, staffing records, and the ISP should line up with what GAMMIS paid.
- **Watch your renewals like a hawk:** a lapsed Secretary of State registration, an expired CLA license, or a Medicaid re-credentialing you missed will stop payments even while the care continues — calendar every renewal date the day you get it.

None of this is exotic; it's the discipline that separates providers who get paid on time from providers who spend their week fighting denials. Build the habit early, while your census is small, and it scales with you.

PART 11

What It Costs, How Long It Takes & Getting Placements

In short: A host home (Path A) can start relatively quickly; a CRA agency (Path B) is a months-long process through the Enrollment Forum, LOI, DBHDD application, CLA licensing, and DCH enrollment. Placements come through the Planning List and your relationships with Support Coordinators.

11.1 Timeline & startup costs

A host home (Path A) can move relatively quickly — screening, training, a home check, and site-specific enrollment through your CRA agency. Becoming your own CRA agency (Path B) is a months-long process: the Enrollment Forum and Letter of Intent (during an open enrollment window), the DBHDD application and any site visit, the CLA licensing of each group home, and DCH Medicaid enrollment. Because a DCH denial carries a one-year wait, plan carefully. Host-home costs are modest — mainly your time, background checks, training, and getting your home ready. A CRA agency adds business formation (\$100 LLC + \$50/yr), insurance, the DDP and staff payroll, the CLA license and fire/physical-plant requirements for each home, and your policies and procedures. Confirm current fees with HFRD and DCH.

11.2 Getting your first placements

Placements don't come from advertising — they come through the Planning List process and your reputation. Here's a simple plan:

- 1. Get known to your Regional Field Office and established providers.** Contact your DBHDD Regional Field Office (dbhdd.georgia.gov/regional-field-offices) and — if you're starting on Path A — connect with established CRA agencies that bring on host homes.
- 2. Build relationships with Support Coordinators.** They help people and families choose providers, and people choose. Be the provider they trust to handle a placement well, communicate, and never leave a shift unstaffed.
- 3. Be responsive and reliable.** Answer the phone, say yes to a good match, handle move-in smoothly, and follow through — one good placement earns the next referral.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your Regional Field Office and the Georgia Collaborative ASO are the center of gravity, with HFRD and your local fire authority close behind. Statewide contacts are the same for everyone; your fire/building offices are local. Here's the list and the scripts.

12.1 Statewide (same for everyone)

For...	Contact
Becoming a provider (enrollment)	Georgia Collaborative ASO — georgiacollaborative.com (New Provider Enrollment Forum & LOI)
Waiver eligibility & local services	Your DBHDD Regional Field Office — dbhdd.georgia.gov/regional-field-offices
Licensing a group home (CLA)	Healthcare Facility Regulation (HFRD) — 404-657-4076 · portal: gahles.dch.georgia.gov
Billing, fee schedule & prior auth	Georgia Medicaid via GAMMIS — mmis.georgia.gov
Crisis line (I/DD)	Georgia Crisis & Access Line (GCAL) — 800-715-4225
Business, EIN & NPI	sos.ga.gov · irs.gov (EIN) · nppes.cms.hhs.gov (NPI)

12.2 The local calls — and exactly what to ask

① Your DBHDD Regional Field Office / a CRA agency — your most important calls

"I want to open a host home / a CRA group home in this area. What's the need, and can you point me to approved CRA agencies?"

"(To a CRA agency) How do you enroll and support host homes, how do you match, and how and when do you pay?"

② Healthcare Facility Regulation (for a group home) — the CLA license

"I want to license a Community Living Arrangement for [3 or 4] adults. How do I start, and what are the fire-safety and physical-plant requirements?"

"Confirm the sprinkler rule for my bed count — my understanding is sprinklers are required only at 7 or more beds."

③ Local fire authority — the fire-safety inspection

"I'm licensing a small residential home for [capacity] adults. What do you require for the fire-safety inspection, and how do I schedule it?"

12.3 Worked examples — Georgia's two biggest metros

Most people start in or near one of these two. Georgia is divided into six DBHDD regions — find yours with the Regional Field Office locator (dbhdd.georgia.gov/regional-field-offices); you're assigned by where you live. Enrollment and the fee schedule are statewide — the local piece is mainly your fire authority and zoning.

- **Atlanta (DBHDD Region 3):** Regional Field Office — DBHDD Region 3, 100 Crescent Centre Parkway, Suite 900, Tucker, GA 30084; (770) 414-3052 (DD Regional Services Administrator: (770) 414-3046), covering metro Atlanta. Enrollment through the Georgia Collaborative ASO; HFRD (404-657-4076) for the CLA; the city (Atlanta) or county fire department for the fire-safety inspection. The quirk: metro Atlanta spans several counties — confirm which county your home is in for the fire inspection and any local zoning.
- **Augusta (DBHDD Region 2):** Regional Field Office — DBHDD Region 2 (East Central Georgia, headquartered near Augusta); find current contact via the Regional Field Office locator. Same statewide Georgia Collaborative ASO enrollment; HFRD (404-657-4076) for the CLA; Augusta-Richmond County fire authority for the fire-safety inspection. The quirk: confirm whether your address is in the City of Augusta / Richmond County or a neighboring county — it changes your fire and zoning contacts.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to become a CRA agency, or can I start smaller?

You can start smaller — Path A lets you become a host home / life-sharing site under an established CRA agency that handles enrollment and billing. It's the fastest way in and the best way to learn the system before you take on your own DBHDD and DCH enrollment (Path B). Many providers grow from A to B.

Why are there two state agencies — isn't this just Medicaid?

Georgia splits the job: DBHDD runs the NOW & COMP waivers, sets the CRA standards, and enrolls and monitors providers, while DCH (Georgia's Medicaid agency) issues your provider number and pays your claims through GAMMIS. So the payment chain is DBHDD enrolls you, DCH credentials and pays you, and HFRD licenses your group home — you generally need all three.

Does the waiver pay for the person's rent and food?

No — the CRA per-diem pays for the SERVICE (the support you provide), not room and board. The resident's rent and food come from their own income, usually SSI. Keep the two completely separate in your agreements and your books, and never bill the person for a covered service; mixing them is a common and serious mistake.

Do I need fire sprinklers in my group home?

Only if the home has seven or more beds. A CRA group home (three or four residents) or a host home (one or two) is well under that, so a sprinkler system generally is not required — but you do need a fire-safety inspection report

from your fire authority, within 12 months, with no violations and showing the capacity load. Confirm anything unusual with your local fire authority.

You can do this

Opening a group home business in Georgia is real work — the waiver system has a lot of moving parts — but it's a real, walkable path, and people across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. Whether you start small with a host home or build your own agency, take it one step at a time: set up the business, learn the CRA standards, enroll with DBHDD and DCH, license the home, ready it, staff it, and build the relationships that bring you placements. Verify the current details with the State as you go, and lean on your Regional Field Office and the Georgia Collaborative ASO — that's what they're there for.

Reminder. Prepared for PolicyReady.org and grounded in the Georgia NOW & COMP 1915(c) waivers, the DBHDD Provider Manual for Community DD Providers, the COMP waiver CRA requirements, and the Georgia Rules & Regulations (including 290-9-37) as of 2026. This is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with DBHDD, the Georgia Collaborative ASO, DCH, HFRD, and qualified counsel before you act.